

HOW TO OPEN A BRAIN INJURY RESIDENTIAL HOME in Colorado

A CDPHE-licensed Assisted Living Residence — funded through the Brain Injury waiver's Supported Living Program.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The Colorado model — read this first

In Colorado, a home that provides room, board, and 24-hour supervised care to adults with brain injury is licensed by the Colorado Department of Public Health and Environment (CDPHE) as an Assisted Living Residence (ALR) under 6 CCR 1011-1 Chapter 7 and C.R.S. Title 25, Article 27. An ALR is a needs-based license — nothing limits it to the elderly — so brain-injury adults are served under the same ALR license, with extra requirements if you run a secured/locked setting for residents who wander. To be paid by Medicaid you take a second step: get your licensed ALR certified as a waiver provider so you can bill the state's Home- and Community-Based Services (HCBS) waivers. This guide walks that whole path, broken down so a first-timer can follow it.

The most important thing to understand about the money in Colorado. Colorado does NOT pay for brain-injury residential care through a generic “assisted living” Medicaid service. The Brain Injury (BI) waiver's residential benefit is the Supported Living Program (SLP) — residential support in a certified assisted-living setting, paid on a 6-tier acuity scale. The separate Alternative Care Facility (ACF) service runs under the Elderly, Blind & Disabled (EBD) waiver, not the BI waiver. Both start from the same CDPHE-licensed ALR, and many operators certify for both so they can take clients from more than one waiver. Part 8 breaks this down — it's the key to getting paid correctly.

Accreditation — strongly recommended, and how we build. Colorado does not require national accreditation to hold an ALR license, but for a brain-injury program it is a real advantage: CARF (which offers a Brain Injury specialty) and The Joint Commission signal quality to case managers, hospital discharge planners (including Craig Hospital), and families, and can strengthen your standing with payers. Every PolicyReady product is built to those standards behind the scenes. Treat accreditation as a goal from day one (see Parts 7 & 9).

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It is written for someone who has never opened a facility — where we name a thing (an agency, a form, a rule, an insurance type), we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with CDPHE, HCPF, your Case Management Agency, and qualified counsel (see Part 11) before you rely on it.

The journey at a glance

Ten milestones take you from idea to a licensed, certified, paid home. Each part below is one rung.

1	Decide this is your business	A licensed, 24-hour brain-injury residential home — durable, census-based revenue (Part 1)
2	Form the business & insure	Entity, EIN, bank, and the right insurance — broken all the way down (Part 3)
3	Secure & prepare the home	Zoning, and the Chapter 7 physical-plant specs (Parts 3–4)
4	Train your administrator	The CDPHE-approved 40-hour ALR administrator training + exam (Part 5)

5	Build & train staff	QMAP medication training, first aid/CPR, background checks (Part 5)
6	Write your policies	P&P Manual + the Chapter 7 operating standards (Parts 4–6)
7	Get the CDPHE license	Letter of Intent → COHFI application → DFPC fire survey → license (Part 6)
8	Certify & enroll for Medicaid	HCBS certification (SLP / ACF) + enroll as a Health First Colorado provider (Part 8)
9	Get your first residents	Case Management Agencies + Craig Hospital / hospital discharge planners (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, all in one place (Part 11)

Rule of thumb. Start your administrator's 40-hour training and your DFPC fire-safety conversation early — those are the two steps that gate your license. Do the cheap, reversible steps (form the entity, sign a contingent lease) first, and get background checks started for everyone right away.

Before you begin — a realistic picture

Be honest with yourself about what this is. You are becoming a licensed health-care operator responsible for vulnerable adults 24 hours a day, seven days a week — not a landlord and not a day program. Expect several months and real money before your first Medicaid dollar arrives: you must form and insure the business, secure and physically prepare a home that meets Chapter 7 and the fire code, put a trained administrator and QMAP-qualified staff in place, get licensed by CDPHE, and only then certify for the waiver and enroll with Medicaid. Two things gate the whole timeline — your administrator's 40-hour course and the DFPC fire path — so start both on day one. The reward is a durable, needs-based business serving a population Colorado's Brain Injury waiver was built for, with census-based revenue that compounds as you fill beds. Read the whole guide once before you spend a dollar; it will change the order in which you do things.

Here is the roadmap. Part 1 defines exactly what this business is and the licensed-ALR-vs-Medicaid-certified distinction. Part 2 names every regulator and what each one does. Part 3 walks you through forming the entity, insurance, zoning, and your NPI. Part 4 covers the building — Chapter 7 physical-plant specs and the DFPC fire code. Part 5 breaks down staff, the administrator, and required training by position. Part 6 is the step-by-step CDPHE licensing path. Part 7 follows a resident from referral to discharge. Part 8 is how you actually get paid — the BI waiver's Supported Living Program, its six tiers, and the claims cycle. Part 9 lays out costs and timelines; Part 10 shows how to fill your beds through Case Management Agencies and Craig Hospital; and Part 11 collects every contact and every “verify” in one place. Work the parts in order and check off each rung as you go.

PART 1

What This Business Is

In short: A CDPHE-licensed Assisted Living Residence for brain-injury adults — room, board, personal care, and protective oversight with 24-hour supervision available. It's a real-estate + 24-hour-care business.

Colorado licenses an Assisted Living Residence (ALR) as a residence that provides room and board plus personal services, protective oversight, and social care, with 24-hour supervision available, for three or more unrelated adults. It's a needs-based license — you're licensed for the level of care your residents need, not for a particular diagnosis — so a home built for adults with brain injury operates under the same ALR license as any other.

Licensed ALR vs. Medicaid-certified ACF/waiver provider

Term	What it means
Assisted Living Residence (ALR)	The CDPHE license itself — you must have this first, whether or not you ever bill Medicaid
Waiver certification (SLP / ACF)	A second step: your licensed ALR is certified by CDPHE and enrolled with Medicaid so you can bill an HCBS waiver
Alternative Care Facility (ACF)	The name for a Medicaid-certified ALR serving EBD/CMHS waiver clients (the ACF service)

Serving residents who wander — the “secure environment”

Chapter 7 doesn't create a separate “memory care” or “secure” license; instead it regulates a secure environment (Part 2.56) — a building, method, or device that prohibits residents' free egress. If any resident is prone to wandering or elopement (common after brain injury), you either run a full secure environment (with added staff training, monthly testing of security equipment, a secured outdoor area, and controlled access to appliances and chemicals) or use an egress-alert device (Part 2.23) that redirects a wandering resident without fully locking the building. Decide this early — it shapes your building and your staffing.

Key terms. CDPHE = Colorado Dept. of Public Health & Environment (your licensor). HCPF = Dept. of Health Care Policy & Financing (runs Medicaid/waivers). ALR = Assisted Living Residence. ACF = Alternative Care Facility (a Medicaid-certified ALR). BI waiver = the Brain Injury HCBS waiver. SLP = Supported Living Program (the BI waiver's residential service). TLP = Transitional Living Program. EBD = Elderly, Blind & Disabled waiver. CMA = Case Management Agency (your intake/referral hub). QMAP = Qualified Medication Administration Person. DFPC = Division of Fire Prevention & Control. NPI = National Provider Identifier. CARF/TJC = the two accreditors.

PART 2

Who Regulates You (and What Each One Does)

In short: CDPHE licenses you, DFPC clears your building for fire safety, HCPF and your Case Management Agency handle Medicaid and referrals, and the Long-Term Care Ombudsman advocates for residents.

CDPHE — Health Facilities & EMS Division (your licensor)

The Colorado Department of Public Health and Environment issues your ALR license, writes the 6 CCR 1011-1 Chapter 7 standards, runs the COHFI online application system, certifies you for Medicaid waiver services, and conducts the pre-licensure and ongoing surveys. Phone 303-692-2000; cdphe.colorado.gov/assisted-living-residences.

DFPC — Division of Fire Prevention & Control (fire & life safety)

DFPC (in the Department of Public Safety) inspects state-licensed health facilities against the NFPA 101 Life Safety Code and issues the Certificate of Compliance you need for licensure. For a brain-injury home whose residents can't all self-evacuate, this matters even more — engage them early. Fire & Life Safety Section: 303-239-4100; dfpc.colorado.gov.

HCPF and your Case Management Agency (who pay you / send residents)

The Department of Health Care Policy & Financing (HCPF) runs Health First Colorado (Medicaid) and the HCBS waivers, and sets the rates. Access for residents runs through regional Case Management Agencies (CMAs) — since July 2024 a single CMA per region handles all waivers, does the functional assessment, and refers members to certified providers. Member line 1-800-221-3943; provider services 1-833-468-0362.

Long-Term Care Ombudsman & Adult Protective Services

The Colorado State Long-Term Care Ombudsman is a free, independent advocate for residents of assisted living and nursing facilities — post the local ombudsman's number for residents (state line 303-862-3524; coombudsman.org). You must also check the Colorado Adult Protective Services (CAPS) system on staff before hire, and report suspected mistreatment.

PART 3

Form Your Business & Secure the Home — Step by Step

In short: *This is where people get stuck. We break each piece down: what it is, where to go, and exactly what to do.*

3.1 Form your legal entity

An “entity” is the legal business (usually an LLC) that holds the license and bank account and shields your personal assets. In Colorado you form it online at the Secretary of State — one of the cheapest and fastest in the country.

- 1. Go to the Colorado Secretary of State.** File online at coloradosos.gov (Business Division, 303-894-2200) — Colorado is online-only for business filings.
- 2. File Articles of Organization (LLC).** The filing fee is \$50. Name the LLC and list a registered agent (receives legal mail — can be you).
- 3. Adopt an Operating Agreement** — the internal document setting out ownership, management, and money. Colorado doesn't file it, but your bank and partners will want it.
- 4. Calendar your Periodic Report.** Every Colorado entity files an annual Periodic Report — the fee is now \$25 (it rose from \$10) — in a window around your formation-anniversary month; late fee \$50. If you use a different public name, file a Statement of Trade Name (\$20).

Colorado has no LLC newspaper-publication step. Unlike some states, Colorado does not make you publish a formation notice — forming the entity is a same-day online filing.

3.2 Get your EIN (federal tax ID)

An EIN is your business's federal tax number — free from the IRS, and you need it for the bank, payroll, and Medicaid enrollment.

- 1. Go to IRS.gov** and search “Apply for an EIN online” (the EIN Assistant). It's free — never pay a third-party site.
- 2. Apply online** with the responsible party's name and SSN/ITIN and your entity info.
- 3. Save the confirmation (CP-575)** — you get the EIN immediately.

3.3 Open a business bank account

What to have ready:

- EIN confirmation from the IRS
- Your Articles of Organization and the entity ID number
- Operating Agreement (LLC)
- A Beneficial Ownership form (owners of 25%+ and a control person)
- Government photo ID, SSN, DOB, home address for each owner/signer
- An opening deposit (often ~\$100)

Where to go: a major bank with lots of Colorado branches — Chase, Wells Fargo, U.S. Bank, FirstBank (a large Colorado-based bank), or a local credit union like Bellco or Ent. Compare monthly fees and how to waive them, transaction/cash-deposit limits, branch access, online bill pay, and whether they offer a business line of credit.

3.4 Get your insurance — what you need, who sells it, and what to ask

An assisted living home cares for vulnerable adults in a building, so you need several kinds of coverage — not one policy:

Coverage	What it protects against	Must-have?
General Liability (GL)	A visitor/family slip-and-fall or property damage on your premises	Yes — foundational
Professional Liability (malpractice / E&O)	Claims about the CARE — supervision, med errors, neglect	Yes — top healthcare coverage
Abuse & Molestation	Allegations of abuse by staff or others (GL)	Yes — high exposure

	excludes this!)	
Commercial Property	The building, contents, equipment; add Business Interruption	Yes if you own; often if you lease
Commercial Auto (incl. hired/non-owned)	Transporting residents; staff driving own cars for work	Yes if you drive residents
Workers' Compensation	Staff injuries (lifting/transferring is high-risk)	Required in Colorado once you have employees
Cyber Liability	A breach of protected health information (HIPAA)	Strongly recommended

Workers' comp is mandatory in Colorado. Unlike Texas, Colorado requires nearly every employer to carry workers' compensation as soon as it has employees — so budget for it from your first hire.

Real places to get it

Buy through an independent agent/broker who accesses specialty carriers. Carriers that write senior-care/residential-care risk include Philadelphia Insurance (PHLY), CNA, Markel, Berkshire Hathaway GUARD, Crum & Forster, Selective, and Nationwide. Specialty programs/brokers for this niche include Caitlin Morgan, Gallagher, AssuredPartners, Professional Insurance Group, and XInsurance. To find a local Colorado agent, use the “Trusted Choice” Find-an-Agent directory at [trustedchoice.com](https://www.trustedchoice.com) (filter healthcare/senior-care), or ask a licensed peer. Verify each carrier writes assisted living in Colorado before relying on a quote.

What to ask the insurance agent

“Do you write assisted living residences / brain-injury homes — which carriers?”

“Is professional liability included — claims-made or occurrence — and what limits?”

“Is abuse & molestation included, at what limit, sub-limit or full?”

“Are GL and professional liability shared limits or separate?”

“What limits does a waiver contract or my landlord/lender require?”

“What will workers' comp cost given my staffing and payroll?”

3.5 Secure & zone the home

- ☐ Call your city/county PLANNING & ZONING office: is a residential assisted living use for [n] residents allowed at this address, or do you need a use permit? Colorado group homes for people with disabilities generally have residential-use protection under state and fair-housing law — ask them to confirm in writing.
- ☐ Confirm the building can meet the Chapter 7 physical-plant specs (Part 4) and be made accessible.
- ☐ Confirm the home can pass a DFPC fire & life-safety review for your resident count (and secure-environment features if you'll lock exits).
- ☐ If leasing, make sure the lease allows your use and resident count, and make it contingent on licensure.

3.6 Map ownership + background checks

Identify every owner and controlling person — you disclose them on the CDPHE application, and every employee must clear a criminal-history check and a CAPS check before contact with residents (Part 5.4).

3.7 Get your NPI (National Provider Identifier)

An NPI is a free national ID payers use to identify you. Your FACILITY needs a Type 2 (Organizational) NPI, and Colorado requires a unique NPI per service location:

1. **Create an I&A account** at nppes.cms.hhs.gov (Identity & Access) for the person who'll manage the number (usually an owner).
2. **Log into NPPES** and start a new application; choose Type 2 (Organizational).
3. **Enter your legal name, EIN, and address** and pick the taxonomy that fits an assisted living / residential facility.

4. Submit — it's free and usually processes in about 10 business days. Save your NPI; you'll need it for Medicaid enrollment (Part 8).

***What this means for you.** The NPI is a bottleneck people forget until it blocks their Medicaid enrollment. It's free and it takes days, not minutes, so apply as soon as your entity and EIN exist — long before you're ready to bill. If you'll ever open a second home, remember Colorado wants a distinct NPI for each service location, so don't reuse one address's number for another building.*

3.8 Foundation-documents checklist

Before you touch the CDPHE application in Part 6, you should be able to lay these on the table. Surveyors, banks, insurers, and Medicaid all pull from this same stack, so assemble it once and keep dated copies in a single binder.

- ☐ Articles of Organization + Colorado entity ID (Secretary of State)
- ☐ Operating Agreement and a current ownership map (everyone at 25%+ and each control person)
- ☐ EIN confirmation letter (IRS CP-575)
- ☐ Type 2 (Organizational) NPI for this exact location
- ☐ Business bank account opened and Beneficial Ownership form on file
- ☐ Insurance binders — GL, professional, abuse & molestation, property, auto, workers' comp
- ☐ Zoning confirmation in writing for your address and resident count
- ☐ A lease (contingent on licensure) or proof of ownership

***Do the reversible, cheap steps first.** Forming the LLC (\$50), getting the free EIN and NPI, and opening the bank account cost little and can't hurt you — do them right away. Sign only a licensure-contingent lease, and don't buy furnishings or hire staff until your zoning answer and DFPC fire path are confirmed. That order protects your cash if the building turns out not to work.*

PART 4

The Home — Physical Plant, Safety & Fire Code (Chapter 7 + DFPC)

In short: Colorado gives you real numbers for bedrooms and bathrooms, defers fire safety to DFPC's Life Safety Code, and adds extra rules if you run a secure setting. Here's what each requirement means and how it's checked.

"Physical plant" just means the building itself and everything fixed in it — the rooms, plumbing, kitchen, and fire systems. CDPHE inspects the health/physical side and DFPC inspects fire & life safety before you're licensed. The rules live in 6 CCR 1011-1 Chapter 7 (interior environment) and DFPC's adopted codes. Below, each requirement is followed by what it means for you.

4.1 Bedrooms & space (Chapter 7, Part 22)

- ☐ Single-occupancy bedroom: at least 100 sq ft of floor space
- ☐ Shared bedroom: at least 60 sq ft PER person
- ☐ No more than two residents per sleeping room (older, pre-1986 buildings may have up to four unless renovated or ownership changes)
- ☐ Each resident has their own bed and personal storage
- ☐ Living/common and dining areas sized for the residents

What this means for you: measure every bedroom before you sign a lease. A single room needs a full 100 sq ft (about 10 by 10); a shared room needs 120 sq ft for two, and you cannot put a third bed in it in a modern building. Brain-injury residents often use wheelchairs or walkers, so leave real turning room beyond the minimum.

4.2 Bathrooms & water (Chapter 7, Part 22)

- **Full bathrooms:** at least one full bathroom for every 6 residents — a full bathroom means a toilet, a hand-washing sink, a mirror, private storage, and a shower.
- **Safety:** grab bars at toilets and in tubs/showers, non-slip surfaces, and features for residents using wheelchairs/walkers/transfer benches.
- **Hot water:** hot water at resident-accessible taps must not exceed 120°F — capped to prevent scalds. Set the water heater and verify with a thermometer; document it.

What this means for you: think like a surveyor walking your bathrooms and kitchen. Water temperature, grab bars, and safe storage of chemicals and medications are the most common early findings. Put a thermometer on the hot-water tap, add grab bars, and lock up chemicals and medications before you apply.

4.3 Fire & the Life Safety Code (DFPC)

- ☐ DFPC inspects the home against the adopted NFPA 101 Life Safety Code for your occupancy and issues a Certificate of Compliance before licensure
- ☐ Confirm the exact Life Safety Code edition, occupancy chapter, sprinkler requirement, and smoke-detector/alarm specs with DFPC for your specific building and bed count
- ☐ Submit the ALR FGI questionnaire and request the fire-safety on-site review
- ☐ A written emergency and evacuation plan that accounts for residents who cannot self-evacuate; fire drills; staff trained to move non-ambulatory residents
- ☐ Keep every fire, building, and health inspection report in one dated compliance binder

Design review can be lighter for smaller homes. Colorado reviews construction/renovation against the FGI residential design guidelines, but a 2025 change (HB25-1213) exempted certain smaller facilities (under 19 beds, no new construction) from some of those requirements. Ask CDPHE and DFPC how the design-review and FGI rules apply to your specific building before you renovate — it can save real money.

4.4 Secure-environment & brain-injury design (why it matters here)

If any resident wanders, Chapter 7's secure-environment rules apply: staff trained on the building layout and residents' mobility, monthly testing of security/egress equipment, a secured year-round outdoor area with a roughly six-foot perimeter barrier and weather-protected seating, and controlled access to appliances and chemicals. Beyond the code, design for brain injury: clear low-clutter paths to reduce falls, consistent labeled locations for personal items (memory support), reduced noise and visual overstimulation, and a quiet space a dysregulated resident can use. This is both good care and part of keeping residents safe.

4.5 Walk your building like a surveyor — a pre-inspection checklist

Before you invite CDPHE and DFPC in, walk every room with this list and a thermometer in hand. Most first-survey findings are small, physical, and cheap to fix if you catch them yourself.

- ☐ Every single bedroom measures at least 100 sq ft; every shared bedroom at least 60 sq ft per person, with no more than two beds
- ☐ At least one full bathroom per 6 residents; grab bars at every toilet, tub, and shower; non-slip surfaces down
- ☐ Hot water at resident taps reads at or below 120°F on a thermometer — and it's written down with the date
- ☐ All chemicals, cleaners, and medications are locked and out of resident reach
- ☐ Smoke detectors, alarms, and (if required) sprinklers are present and tested; exits are clear and marked
- ☐ The written emergency and evacuation plan names who moves each non-ambulatory resident, and drills are logged
- ☐ If any exit is secured: egress/security equipment tested monthly, secured outdoor area with ~6-ft barrier and seating
- ☐ One dated compliance binder holds every fire, building, and health inspection report

Practical tip. Take date-stamped photos of your grab bars, locked med storage, water-temperature reading, and exit signage before the survey. If a surveyor questions something you've since adjusted, a dated photo showing the condition on inspection day is the cheapest evidence you can carry.

PART 5

Staff, the Administrator & Required Training — by Position

In short: Each role has its own age, training, and screening rules. Here they are, one by one — and the 40-hour administrator training and your QMAP medication staff are the pieces to start early.

5.1 Your ALR administrator (Chapter 7, Part 6)

Every ALR must have a qualified administrator (who may be you). Here's exactly what that takes:

1. **Meet the floor.** At least 21 years old, with a high-school diploma or equivalent.
2. **Meet one experience pathway.** For example: one year supervising personal-care/ADL delivery; an active nursing-home administrator license; an RN plus 6 months (or LPN plus 1 year) of healthcare experience; a health/human-services degree plus 1–2 years; or a bachelor's plus 2 years of healthcare experience — see Part 6 for the full list.
3. **Complete the 40-hour ALR administrator training BEFORE you take the role.** 20 of the 40 hours cover Colorado regulations; the rest cover operations, management, physical plant, and resident care. Approved vendors include the Colorado Gerontological Society and the Center for Elder Care Training & Education.
4. **Pass the competency exam.** A competency test is required, and the result is kept in your personnel file. (Colorado does not issue a separate state administrator license — the requirement is this approved course plus the exam.)
5. **If you step in as an interim,** complete the training within 30 days of appointment.
6. **Keep up your continuing education** as Chapter 7 requires (confirm the current annual hour count with CDPHE).

Start this early. The 40-hour course plus the exam take real time to schedule and finish, and you can't take the administrator role without them. Line up an approved course before you apply. CDPHE 40-hour administrator training page; Colorado Gerontological Society (senioranswers.org).

5.2 Medication — the QMAP requirement (Chapter 24)

Colorado does not let just anyone pass medications. A Qualified Medication Administration Person (QMAP) — a staff member who has passed a state-recognized competency course and is at least 18 — must administer medications in an ALR, and you keep a QMAP available whenever medications are needed. Colorado has tightened QMAP rules recently (a designated QMAP supervisor is now required, and QMAPs certified before 2017 had to retrain by January 1, 2025), so verify current requirements. Plan to have several QMAP-qualified staff so you're always covered.

5.3 Direct-care staff & coverage (Chapter 7, Parts 7–8)

- **Presence:** whenever a resident is present, at least one qualified staff member able to respond to an emergency must be on-site.
- **First aid & CPR:** at all times, at least one on-site staff member with current first aid, and at least one with current CPR and obstructed-airway certification from a nationally recognized program.
- **Overnight:** Colorado does not impose a blanket awake-overnight rule, but between 10 p.m. and 6 a.m. staff must make at least one safety check of all consenting residents — and a brain-injury home's acuity and any secure-environment or wandering risk often make awake overnight coverage necessary. Document a staffing plan a surveyor can see meets your residents' needs.
- **Orientation:** before working, staff complete orientation covering care duties, infection control, basic first aid, emergency response, mandatory reporting, resident rights, and your policies.
- **Dementia/behavior training:** at least 4 hours of competency-based dementia training initially, plus at least 2 hours every 2 years, and role-specific training in behavior management, communication, fall prevention, and safe lifting.

5.4 Brain-injury competence & background checks

Colorado's rules require dementia and behavior training but do not spell out brain-injury-specific competencies — so make them part of your own program: cognitive stimulation, wander- and personal-safety, responding to sudden or dysregulated behavior, and communication strategies for cognitive/speech impairment. On screening, run these before any resident contact:

1. **Run a criminal-history record check** before hire — a CBI (Colorado Bureau of Investigation) name-based check for people who've lived in Colorado 3+ years, or checks from each prior state of residence for newer arrivals.
2. **Check the CAPS system** — query the Colorado Adult Protective Services data system (per C.R.S. § 26-3.1-111) on every applicant.
3. **Fingerprint where required** — owners/controlling persons and certain roles route through the Colorado Applicant Background Service (CBI + FBI). Budget roughly \$16–\$40 depending on whether the FBI check is included; confirm the current fingerprint scope for staff vs. owners with CDPHE.
4. **Document everything** — keep the dated results in each employee file; a surveyor will ask to see them.

What every personnel file must hold

A surveyor will pull employee files at random and expect each to be complete on the day the person started working. Build one template file and use it for every hire so nothing is missing.

- ☐ Application, job description signed by the employee, and proof of age (18+ for QMAP; 21+ for the administrator)
- ☐ Dated criminal-history (CBI/out-of-state) result and CAPS check result — both completed before any resident contact
- ☐ Fingerprint-based results where required for owners/controlling persons and certain roles
- ☐ QMAP certificate (for med-passing staff) and current first aid / CPR / obstructed-airway cards
- ☐ Orientation record and the dementia/behavior training hours (4 hours initially, 2 hours every 2 years)
- ☐ Your own brain-injury competency training: cognitive support, wander safety, de-escalation, communication
- ☐ Signed acknowledgment of resident rights, mandatory-reporting duties, and your policies

Build depth into your schedule. *Because Colorado wants first aid and CPR coverage on-site at all times, a QMAP available whenever meds are due, and often awake overnight staff in a brain-injury home, one certified person per role is not enough. Cross-train several staff to QMAP and CPR so a single call-out never leaves you out of compliance — and log a staffing plan a surveyor can read.*

PART 6

Get Your CDPHE License — Step by Step

In short: *File a Letter of Intent, complete the application in COHFI, pass the DFPC fire survey and the CDPHE pre-licensure inspection, and receive your ALR license. Start early — submit about 90 days before you want to open.*

6.1 Before you apply

- ☐ Entity formed, EIN, NPI, insurance bound (including workers' comp)
- ☐ Home secured and brought up to Chapter 7 specs; DFPC fire path confirmed
- ☐ Administrator identified and enrolled in the 40-hour training
- ☐ QMAP-qualified staff lined up; policies & procedures written (your P&P Manual)
- ☐ Background-check system in place for staff

6.2 The application

- 1. File a Letter of Intent (LOI).** You request the application by filing an LOI with CDPHE Health Facilities.
- 2. Complete the application in COHFI.** CDPHE's online health-facility licensing system — submit the application and the non-refundable fees.
- 3. Pay the fees.** The exact ALR initial-license amount (and any per-bed component) is published in CDPHE's health-facility fee schedule (updated each July 1) — pull the current figure or call 303-692-2000; don't rely on a guessed number.
- 4. Clear design/plan review.** Construction/renovation is reviewed against the FGI residential design guidelines (with the smaller-facility exemptions from Part 4).
- 5. Pass the DFPC fire & life-safety review.** Submit the ALR FGI questionnaire and get your Certificate of Compliance (DFPC 303-239-4100).
- 6. Pass the CDPHE pre-licensure survey.** An on-site inspection/fitness review before the license is issued.

What CDPHE will ask you for:

- Your entity and ownership; the administrator's qualifications and training; your policies & procedures; floor plans and life-safety documentation; and your background-check process.

6.3 Timeline

Submit your application and fees at least 90 days before your target opening. CDPHE does a completeness review and you cure any defects (often on a 14-day response window). Between the design/plan review, the DFPC fire survey, and the pre-licensure inspection, plan for a few months end to end.

Ask before you file. *Call CDPHE Health Facilities at 303-692-2000 to confirm the current LOI process, the fee amount, and exactly which attachments your application needs — and DFPC at 303-239-4100 about the fire path for your building. A complete first submission saves weeks.*

PART 7

The Resident's Journey — Referral to Discharge

In short: A waiver client is assessed and referred by their Case Management Agency; you confirm the fit, admit under a residency agreement, deliver and document the support plan, and plan for the least-restrictive setting. Here is the whole journey, stage by stage.

Stage 1 — Referral comes in

A referral comes from a Case Management Agency case manager, or a hospital or rehab (e.g., Craig Hospital) discharge planner (Part 10). Someone asks whether you have a bed for a specific person on the BI (or EBD) waiver.

What you do:

- Take the basics (age, diagnosis, waiver, acuity/tier, target move date). Ask for the person's assessment (ULTC 100.2) and support plan. Tell them your certification (SLP and/or ACF) and current openings.

Stage 2 — Pre-admission screening (can you safely serve this person?)

Before you say yes, confirm the resident fits an ALR, your capacity, and your abilities. Their needs can't exceed what an ALR can provide (an ALR is not a nursing facility), and if they wander you must have the secure-environment or egress features to keep them safe.

What to review:

- Diagnoses and behaviors (aggression, elopement/wandering, swallowing/choking risk); mobility and transfer needs; medications and any special monitoring; and whether your trained, QMAP-covered staffing can meet all of it safely. When in doubt, ask the case manager and the resident's physician.

Stage 3 — The Case Management Agency sets the plan

For waiver clients, the CMA — not you — confirms the qualifying diagnosis, runs the functional assessment (ULTC 100.2) to establish the institutional level of care, sets the acuity tier, and builds the support plan. The member has free choice among certified providers, and the CMA authorizes the service and tier that drive your payment.

Stage 4 — Admission & the residency agreement

On move-in, the resident (or their representative) signs a residency agreement covering services, resident rights, fees, who pays room and board, and how residency can end. Give a rights orientation and open a resident record.

The resident record must include:

- Health information and diagnoses; a current medication list with allergies; advance directives / medical power of attorney; physician and case-manager contacts; emergency contacts and the representative; and the signed agreement and consents.

Stage 5 — Build the service/support plan (the care roadmap)

Within your policy timeframe, develop and follow the resident's care plan — the day-to-day plan for THIS person, aligned to the CMA's support plan. For a brain-injury resident it typically covers cognitive-stimulation activities, personal- and wander-safety measures, help with the activities of daily living, medication administration by a QMAP, nutrition and any special diet, and behavior supports. Coordinate with the physician and CMA, and have the resident/representative sign off.

Stage 6 — Daily life & care

Deliver the plan: structured daily routines, ADL support, cognitive and social activities, community outings as appropriate, meals, and safe QMAP medication practices — all mapped to the resident's goals. Maintain your required coverage and overnight safety checks, and document progress notes so you can show the plan is being followed and justify the acuity tier.

Stage 7 — Reviews, incidents & rights

Review and update the care plan as needs change and whenever the CMA reassesses (which can change the tier and your rate). Run your quality program with incident reports; protect resident rights; and report any suspected

abuse, neglect, or exploitation to Adult Protective Services immediately. Everything is documented and followed up.

Stage 8 — Discharge & transition

When a resident's needs change or goals are met, coordinate a safe transition with the CMA and family — to a less-restrictive setting, a higher level of care (a nursing facility if they can no longer be safely served in an ALR), or the community — following your termination-of-residency policy and giving proper written notice. Send the receiving provider the records they need, and document the transition.

The documentation that protects you at every stage

The through-line of the whole journey is paper. Good records keep residents safe, satisfy surveyors, and — because Medicaid pays by acuity tier — directly justify the rate you bill. If it isn't written down, to a surveyor it didn't happen.

- ☐ Progress notes that show the care plan is actually being delivered day to day
- ☐ Notes describing the behaviors, mobility, and supervision that justify the resident's assessed tier
- ☐ Every incident report, with the follow-up and any notification to the CMA, physician, or APS
- ☐ Medication administration records (MARs) completed by the QMAP each pass
- ☐ Care-plan reviews dated to each change in need and each CMA reassessment
- ☐ Signed residency agreement, consents, and rights orientation — kept current

What this means for you. When the CMA reassesses a resident, the tier — and your daily rate — can move up or down. Contemporaneous notes that document a resident's real supervision and behavior needs are what support a higher tier at reassessment; thin charting quietly costs you revenue and can leave a rate unsupported on audit. Treat documentation as a billing function, not just a clinical one.

PART 8

How You Get Paid — the BI Waiver (SLP), Step by Step

In short: License your ALR, certify it for the waiver service (SLP for brain injury; ACF for EBD), enroll as a Health First Colorado provider, and bill the per-diem. Medicaid pays for the care — never room & board. Verify each resident's waiver and tier before admission.

Summary of the sources:

Source	What it is	Key fact
BI waiver — SLP (T2033)	Brain Injury waiver residential support	Pays a per-diem on a 6-tier acuity scale; the main route for brain-injury residents
EBD waiver — ACF (T2031)	Alternative Care Facility service	A separate per-diem for elderly/physically-disabled residents; certify for it too, to widen your census
Private pay	Family/self-pay	Always covers room & board; may cover all care for non-waiver residents
Veterans (VA)	Aid & Attendance, VA residential care	May help fund a veteran's care — verify eligibility

8.1 Understand which service you bill

This is the Colorado-specific step people get wrong. The Brain Injury waiver's residential benefit is the Supported Living Program (SLP), paid on six acuity tiers — NOT the “ACF” service. The ACF service belongs to the EBD and CMHS waivers. Both start from your licensed ALR, so many operators get certified for BOTH (SLP and ACF) and take clients from more than one waiver. There's also a time-limited Brain Injury Transitional Living Program (TLP) for residential rehab/transition. Decide which certifications you want before you enroll.

8.2 License, certify, get your NPI, then enroll

- 1. License the ALR first (Part 6).** You cannot be certified for a waiver service until CDPHE has licensed the building.
- 2. Get HCBS-certified through CDPHE.** Submit a Letter of Intent for the specific service — SLP (specialty code 680) for brain injury, ACF (code 602) for EBD/CMHS — pass an initial zero-deficiency survey, and receive your Certification & Transmittal (C&T) document. A separate certification is required per location.
- 3. Have your Type 2 NPI ready (Part 3.7)** — Colorado requires a unique NPI per service location.
- 4. Enroll as a provider through Gainwell.** Colorado's fiscal agent is Gainwell Technologies; register and submit the Provider Enrollment Application on the Provider Web Portal, attaching your CDPHE C&T. Provider Services: 1-833-468-0362.
- 5. Revalidate periodically** to keep your enrollment active.

8.3 The rates & what's covered (2025–2026)

SLP (the BI-waiver residential service, code T2033) is billed per day on six acuity tiers set by the client's assessment. Current daily rates (outside Denver County / Denver County):

SLP tier	Outside Denver	Denver County
Tier 1	\$241.05	\$245.58
Tier 2	\$281.83	\$288.50
Tier 3	\$314.03	\$322.05
Tier 4	\$376.21	\$387.01
Tier 5	\$414.29	\$426.90
Tier 6	\$460.29	\$475.38

For comparison, the ACF service (EBD/CMHS, code T2031) pays about \$105.38/day (\$111.07 in Denver), and the time-limited Brain Injury Transitional Living Program (TLP, T2016) pays about \$762/day. Pull the current figures

from HCPF's rate schedule before you rely on them; higher acuity = higher tier = higher pay, so accurate documentation of each resident's needs matters.

Room & board is NOT covered by Medicaid. The waiver pays only the care/service component. The resident pays room and board from their own income, capped by state rule: effective January 1, 2026 the maximum room & board is \$810/month, and the resident keeps a Personal Needs Allowance (minimum \$184, maximum \$435.46/month), against a 2026 maximum SSI of \$994/month. You collect room & board directly from the resident by a written monthly due date. Build this into your admission and your residency agreement.

How you actually bill (the claims cycle)

1. **Verify eligibility & the authorization** — confirm the member's waiver enrollment, the authorized service (SLP/ACF), and the tier the CMA set before and during the stay.
2. **Submit the claim** — per diem (1 unit = 1 day) through the Gainwell Provider Web Portal, using your NPI and the authorized service code and tier.
3. **Watch timely filing** — Colorado Medicaid allows 365 days from the date of service, but you must re-submit a pending claim every 60 days to hold that status.
4. **Collect room & board** from the resident by your written due date, separate from the Medicaid claim.

Ask HCPF / your CMA:

- Which service and tier this resident is authorized for; what documentation supports each tier; how a reassessment changes the tier and rate; the current SLP/ACF/TLP rates; the exact room-and-board cap and PNA; and the claims portal and timely-filing details.

8.4 Private pay & veterans

- **Private pay:** non-waiver residents (and every resident's room & board) pay privately — Colorado assisted living averages roughly \$5,000/month and a specialized brain-injury home typically prices above that; set a clear rate and a written residency/financial agreement, reviewed by counsel.
- **Veterans:** VA Aid & Attendance (an add-on to the VA pension) can help fund a veteran's care — 2025–2026 maximums are about \$2,424/month for a single veteran and about \$1,558/month for a surviving spouse (verify on VA.gov). The VA Polytrauma/TBI System of Care is also a resource for veterans with brain injury.

PART 9

What It Costs & How Long It Takes

In short: Budget the home, the 40-hour administrator training, QMAP and staff training, background checks, and the CDPHE fees — and consider accreditation. Plan a few months through licensing, then certification.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
The home	Lease deposit + first month (or purchase); accessibility & secure-environment mods; furnishings	Varies by market/size
Administrator training	CDPHE-approved 40-hr course + competency exam	Several hundred \$
Staff & QMAP training	QMAP course per med-passing staff; first aid/CPR; orientation	Per person
Background checks	CBI/CAPS name checks (low cost); fingerprints ~\$16–40 where required	Per person
Insurance	GL + professional + abuse + property + auto + workers' comp (required), annual	Get quotes
CDPHE license & fire	Initial ALR license fee (per fee schedule) + DFPC review	Confirm current amounts
Business setup	SOS \$50 + \$25/yr periodic report, EIN (free), NPI (free), bank, legal	~\$75 + legal
Policies	P&P Manual + Forms Packet	One-time
Accreditation (optional but recommended)	CARF or Joint Commission (see 9.2)	Low-to-mid five figures over the cycle

9.2 If you pursue accreditation (recommended for brain injury)

- **CARF:** accredits brain-injury programs with a Brain Injury specialty. The survey fee is quote-based (commonly cited around a ~\$995 application fee plus roughly \$1,525 per surveyor per day, on a 1- or 3-year term) plus an annual conformance report — request a current quote. (888) 281-6531, carf.org.
- **The Joint Commission:** offers Behavioral Health Care & Human Services accreditation; you pay an annual fee across the 3-year cycle plus a survey-year fee, both priced from your services and census. Request a quote: (630) 792-5000, jointcommission.org.
- **It's staged:** both require you to be operating and serving residents before survey, then an initial on-site survey, then a decision. Plan roughly 12–18 months. The VA's brain-injury programs are CARF-accredited, which tells you what payers value.

9.3 Timeline

Early	Administrator course & DFPC	Enroll in the 40-hr training; confirm the fire path with DFPC
Weeks	Business & home ready	Entity, insurance (incl. workers' comp), home to Chapter 7 specs, zoning
Weeks	Staff & policies ready	QMAP staff, first aid/CPR, background checks, P&P adopted
~90 days+	LOI → COHFI → surveys → license	Apply → design review → DFPC fire survey → pre-licensure survey → license
Then	Certify & enroll	HCBS certification (SLP/ACF) + Gainwell provider enrollment
Ongoing	Referrals & full census	Case Management Agencies + Craig / hospital discharge planners

PART 10

Getting Your First Residents

In short: Waiver clients reach you through their Case Management Agencies, and through hospital and rehab discharge planners — Craig Hospital is the marquee brain-injury pipeline in Colorado. Build those relationships before you open.

10.1 The referral sources

- **Case Management Agencies (CMAs):** once you're a certified, enrolled provider, the regional CMA's case managers authorize services and match members needing residential care to providers (members have free choice). Tell the CMA(s) covering your area your openings, certifications (SLP/ACF), and brain-injury capability.
- **Craig Hospital & hospital discharge planners:** the biggest pipeline for brain injury — the trauma centers and neuro-rehab hospitals below discharge patients who need a residential home. Build the relationship with their case-management/social-work teams; leave a one-pager and ask how to get on their placement list.
- **Brain Injury Association of Colorado (BIAC):** its free Resource Navigation program connects survivors and families to providers — a two-way relationship. 303-355-9969 / 1-800-955-2443, biacolorado.org. The state's MINDSOURCE Colorado Brain Injury Program funds navigation services.
- **2-1-1 Colorado & ADRCs:** the statewide information & referral line (dial 211) and the Aging & Disability Resource Centers route people needing services — get on their radar.

10.2 Craig Hospital & trauma / neuro-rehab centers (verify before relying)

Metro	Program (referral source)	Contact (verify)
Englewood	Craig Hospital — the flagship CO brain-injury & SCI rehab	303-789-8000 (Admissions via main line)
Denver	Denver Health (Level 1 trauma); UCHHealth Univ. of CO Hospital (Aurora, Level 1)	303-436-6000; 720-848-0000
Denver	HealthONE Swedish (comprehensive stroke/neuro) + Swedish Rehab	303-788-5000
Colo. Springs	UCHHealth Memorial Hospital Central (Level 1 trauma)	719-365-5000
N. Colorado	UCHHealth Medical Center of the Rockies (Loveland, Level 1)	970-624-2500
Grand Junction	Intermountain Health St. Mary's Regional (Level 2 trauma)	970-298-2273
Pueblo	UCHHealth Parkview Medical Center	719-584-4000

Ask each referral source:

- How do you decide where to place brain-injury patients who need a residential home? What do you need from me to be on your placement list (license, certification, capability, availability)? Who is the discharge planner / social worker / case manager I should stay in touch with?

10.3 Your regional Case Management Agencies (Denver-metro examples)

CMA	Counties	Phone
Rocky Mountain Human Services	Denver, Adams	303-636-5600
Developmental Pathways	Arapahoe, Douglas, Elbert	303-360-6600
Jefferson County DHS	Jefferson, Clear Creek	303-271-4216
A&I Avenues	Boulder, Broomfield, Gilpin	303-439-7011
The Resource Exchange	El Paso, Park, Pueblo, Teller	719-380-1100

Find the CMA for any county in the HCPF Case Management Agency directory. These are the people who send you residents — introduce yourself early.

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, the administrator & the building

Who	Contact	What to verify / ask
CDPHE — Health Facilities & EMS Division	303-692-2000 · cdphe.colorado.gov/assisted-living-residences	The LOI & COHFI application; the current ALR license fee; the pre-licensure survey; HCBS certification
DFPC — Fire & Life Safety	303-239-4100 · dfpc.colorado.gov	The Life Safety Code edition/occupancy for your building; sprinklers; the Certificate of Compliance
CDPHE 40-hr administrator training	cdphe.colorado.gov/40-hour-administrator-training	The approved course + competency exam; the CE requirement
QMAP program (CDPHE)	cdphe.colorado.gov (QMAP)	Current QMAP course, supervisor rule, and re-training requirements
City/County Planning & Zoning	your jurisdiction	Allowed use at your address; group-home protection — in writing
CBI / CAPS	cbi.colorado.gov · CAPS via CDHS	Criminal-history & CAPS checks; fingerprint scope for staff vs. owners

Business, insurance & billing IDs

Who	Contact	What to verify / ask
Colorado Secretary of State	303-894-2200 · coloradosos.gov	Forming your LLC (\$50); the \$25 periodic report; trade name
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
NPPEs / CMS	nppe.cms.hhs.gov	Your Type 2 NPI (one per location) and the right taxonomy
Insurance agent (Trusted Choice)	trustedchoice.com ; carriers in Part 3.4	Coverages & limits; abuse & molestation; workers' comp cost
Business bank	Chase/Wells/U.S. Bank; FirstBank; a local credit union	Account fees, limits, business line of credit

Funding & referrals

Who	Contact	What to verify / ask
HCPF — provider enrollment (Gainwell)	1-833-468-0362 · hcpf.colorado.gov/provider-enrollment	Enrolling your NPI; the C&T; revalidation
HCPF — HCBS policy / rates	HCPF_HCBS_Questions@state.co.us · hcpf.colorado.gov	SLP vs ACF certification; the current tier rates; the room-and-board cap & PNA
Health First Colorado — member line	1-800-221-3943 (Relay 711)	How members enroll and reach case management
Your Case Management Agency	HCPF CMA directory (Part 10.3)	Which CMA covers my area; how referrals & authorizations work; the tier assessment
Accreditor — CARF / TJC	CARF (888) 281-6531 · TJC (630) 792-5000	Cost quote; the operating-first requirement; Brain Injury specialty

Advocacy, the state BI program & referral partners

Who	Contact	Role
Colorado State Long-Term Care Ombudsman	303-862-3524 · coombudsman.org	Independent resident-rights advocate — post the local number for residents
Adult Protective Services (CAPS / CDHS)	report via your county APS	Mandatory abuse/neglect/exploitation reporting; the CAPS check
Brain Injury Association of Colorado	303-355-9969 · 1-800-955-2443 · biacolorado.org	Resource navigation; survivor/family support; two-way referrals
MINDSOURCE — CO Brain Injury Program	mindsourcecolorado.org	The state brain-injury program funding

		navigation & services
Craig Hospital	303-789-8000 · craighospital.org	The flagship brain-injury rehab — your key discharge-planning relationship

You can do this

Opening a Colorado brain-injury residential home is a real build — a licensed ALR to Chapter 7 specs, a trained administrator, QMAP-qualified staff, and vulnerable residents — but it's a defined path, and Colorado's Brain Injury waiver is built for exactly these residents. Take it one rung at a time: start your administrator's 40-hour course and your DFPC conversation, form the business, prepare the home and team, get licensed by CDPHE, certify for the SLP (and ACF) waiver services and enroll with Health First Colorado, and earn referrals from the Case Management Agencies and Craig Hospital. Verify each rate and contact as you go, and lean on the people in Part 11.

Reminder. Prepared for PolicyReady.org and grounded in 6 CCR 1011-1 Chapter 7, C.R.S. Title 25 Art. 27, and current HCPF/HCBS information as of 2026. This is general information, not legal, tax, or billing advice.

Requirements, fees, rates, and contacts change — verify each with CDPHE, DFPC, HCPF, your Case Management Agency, and qualified counsel before you rely on it. Confirm your service certification (SLP/ACF), the current tier rates, and the room-and-board cap before you admit and bill.